

Prior Authorization Policy: Elective Knee Replacement (DRG 470/469)

Policy Number: POL-ORTHO-2024-001 | Category: Prior Authorization | Department: Medical Management

1. PURPOSE

This policy establishes the prior authorization requirements and medical necessity criteria for elective total and partial knee replacement procedures (arthroplasty) billed under DRG 470 (Major Joint Replacement Lower Extremity without MCC) and DRG 469 (with MCC).

2. SCOPE

Applies to all Payer Co commercial, Medicare Advantage, and Exchange plan members requiring elective orthopedic surgical intervention for osteoarthritis of the knee.

3. DEFINITIONS

Total Knee Arthroplasty (TKA):

Surgical replacement of the knee joint with a prosthetic implant. CPT 27447 (total), CPT 27446 (partial/unicompartmental).

DRG 470:

MS-DRG grouping for Major Joint Replacement Lower Extremity without Major Complication or Comorbidity (MCC). Expected length of stay: 2-4 days.

Primary ICD-10 Indicators:

- M17.11 - Primary osteoarthritis, right knee
- M17.12 - Primary osteoarthritis, left knee
- M17.31 - Secondary osteoarthritis, right knee

4. PRIOR AUTHORIZATION REQUIREMENTS

4.1 Authorization is REQUIRED for:

- All elective TKA procedures regardless of facility type
- Admission must be authorized prior to scheduled date of service
- Authorization period: up to 4 days inpatient (standard TKA without complications)
- Authorization number must appear in FL 63 of UB-04 claim form

4.2 Medical Necessity Criteria (ALL must be met):

- Documented moderate-to-severe knee OA (Kellgren-Lawrence Grade III or IV)
- Functional limitation documented in medical record (pain at rest, limited ROM)
- Failure of conservative therapy (minimum 3 months PT, NSAIDs, or injections)
- BMI < 40 or documented weight management program if BMI 40-50
- Medically stable for general or spinal anesthesia

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5. AUTHORIZATION DURATION AND EXTENSION

Standard authorization covers 3 days inpatient post-procedure.

Day 1 authorization covers day of surgery through expected Day 3 discharge.

Extension criteria - clinical review required for stays beyond 4 days:

- Unexpected surgical complication (infection, DVT, PE)
- Hemodynamic instability requiring monitoring
- Inability to participate in PT (pain >7/10 uncontrolled)
- New cardiac event post-operatively

NOTE: Unauthorized days beyond the approved authorization will be subject to retrospective review and may result in denial. Providers must call 1-800-XXX-XXXX for same-day extension requests.

6. AUDIT TRIGGERS

The following patterns trigger automatic claims audit routing:

- Actual LOS exceeds authorized days by 1+ day without extension
- Authorization dates do not overlap with claim statement dates (FL 6)
- Rendering provider NPI on claim differs from requesting provider NPI on auth
- Total charges exceed \$100,000 for DRG 470 without MCC

7. CODING GUIDANCE

Principal diagnosis must be the condition that required the admission after study.

Secondary diagnoses (E11.9, I10) should be coded when documented and treated.

Z96.641/Z96.642 (presence of joint implant) should be coded as secondary if applicable.

Discharge status must accurately reflect disposition (FL 17).

8. REFERENCES

CMS ICD-10-CM Official Guidelines for Coding and Reporting FY2024

InterQual Criteria: Acute Care, Musculoskeletal - Knee Replacement

Milliman Care Guidelines: Orthopedic Surgery

CMS MS-DRG Grouper and Definitions Manual Version 41